IMC - Internal Monitoring Checklist

Review identification, purpose and classification guide

IMC · v1.5

Study / protocol	CIP version	Site / code	IMC ID
IMP ID / version applied	**Review type** ☐ Scheduled ☐ Focused ☐ Triggered	**Review period / data cut-off**	**Date of review**
Trigger or reason (if not scheduled)		**Reviewer / role**	**Controlled location**

A Purpose and use

The IMC records one internal review of the site's own controls, run as the Internal Monitoring Plan (IMP) sets out. For each check in scope the reviewer rates three things separately: whether the control was done (execution), whether the records show it (evidence) and whether it worked (effectiveness). The IMC cites the IMP version it followed; it does not change the plan's rules after seeing the results.

It refers to records by ID and does not copy their content. It does not judge a single participant, image or event, which stay in their own records, and it does not replace sponsor monitoring, safety reporting or CAPA.

Unit of analysis	Decision supported	Inputs	Outputs
One site, one review period or trigger, and the checks in scope.	Whether each control was done, evidenced and effective, and what must be corrected, sent on or verified next.	IMP, records named for each check, previous IMC and open actions.	Findings and actions (G), routes to the PI Oversight Template (POT), SRR, CAPA and PIR, and the effectiveness check (I).

B Classification guide

Rate the three dimensions separately for each check in E. A negative or uncertain rating always needs a finding, limitation or action.

Rating	Use when	Minimum response
Execution		
Complete	All planned activity for the period was done, even if it found a problem.	Rate evidence and effectiveness.
Partial	Part of the planned activity was done, or a smaller sample than the IMP without approval.	Record what is missing; action with owner and date.
Not performed	The activity was due and not done, or cannot be confirmed.	Action with owner and date; consider escalation.
Not applicable	The check does not apply to this phase or scope.	Reason in D.
Evidence		
Adequate	Records confirm what was done and what was found.	None.
Partial	Records confirm only part of it, or are scattered or late.	Name the missing record; evidence action.
Insufficient	Records missing, contradictory or not retrievable.	Action; effectiveness is not assessable.
Not applicable	Only when execution is also not applicable.	Reason in D.
Effectiveness		
Effective	The control works and no relevant signal is open.	None.
Observation	Minor, isolated point with no sign of control failure.	Record; action only if follow-up is needed.
Not effective	Recurrence, material gap, failed response or open signal.	Finding, action, owner, date and route.
Not assessable	Cannot be judged from the records available.	State the limitation; action to complete it.

B Classification guide (continued): combinations that change the response

Execution · Evidence · Effectiveness	Meaning	Minimum response
Complete · Adequate · Effective	The control works and can be shown.	Record the result.
Complete · Adequate · Not effective	The control was done and recorded but did not prevent or catch the problem.	Finding and action; consider CAPA or SRR.
Complete · Insufficient · Not assessable	Said to be done, but the records cannot show it.	Evidence action; re-rate at the next review.
Not performed · Adequate	Only when records show that the activity did not happen. A record of absence never shows execution.	Action with owner and date.

C What each check looks for

A general guide. The site-specific criteria are in IMP section E and prevail over this table.

#	Check	Execution means	Evidence may include	Effective when
1	**Participant entry: consent and eligibility**	Consent and eligibility of the participants included in the period were checked.	Signed consent forms, version log, eligibility checklist, PSEAR.	No procedure before consent; eligibility signed before surgery; any gap reported as a deviation.
2	**Delegation and training**	Study tasks done in the period were matched to the delegation log and training records.	Delegation log, training records, task records (ICDL, PFVR).	No task done before delegation and training; gaps closed and routed.
3	**Sterile release to use**	Release, package, expiry and storage checks for the period were reviewed.	ICDL, release record, storage and expiry record, substitution record.	Release status and any breach or substitution reconciled and routed.
4	**Implant traceability and component reconciliation**	Planned configuration, implanted components and accountability were compared.	ICDL, implant labels, accountability log, operative note, CRF.	Lot, UDI and configuration agree, or each discrepancy is explained and routed.
5	**Instrumentation and operating-room readiness**	Readiness, compatibility and malfunction records were reviewed.	ICDL, room readiness record, deficiency record.	Missing, incompatible or faulty equipment recorded and dealt with.
6	**Operative documentation**	Operative notes were read for configuration, decisions, substitutions and complications.	Operative note, ICDL, SART reference, CRF query.	Notes complete enough to read safety and outcome later.
7	**Imaging usability and timing**	Imaging timing, views and usability were reviewed.	IQL, imaging source, PFVR, repeat record.	Each unusable or late set has a disposition, repeat or limitation.
8	**Follow-up execution**	Follow-up outputs were reviewed for timing and components.	PFVR, FUCR, source and CRF.	Missing components have a cause and a recovery route.
9	**Endpoint data agreement**	A sample of endpoint-critical fields was compared with the source.	Source record, CRF entry, comparison sheet, query log.	Sampled fields agree, or each discrepancy is corrected and explained.
10	**Timeliness and closure**	Items due in the period were checked against their deadlines.	Query log, action log, CAPA log, time stamps.	Backlog visible, owned and inside the agreed limit.
11	**External context**	The planned search of external information was done.	Search record, field notice, sponsor letter, PI note.	Relevant information read against local data and sent on when needed.
12	**Complication monitoring**	Planned CSST reviews and their routing were checked.	SCML, CSST, SART.	Reviews on schedule; each signal has a route; no event left unindexed.

D Checks in this review and what was reviewed

Scope and sample follow the IMP; a different rule is also entered in IMP section I, never applied silently. Write how many units were reviewed out of how many, and the records and range used. A check left out needs the reason and who approved it.

#	Check	In scope?	Sample reviewed (n of N)	Records, ID range and cut-off	If not in scope or sample changed: reason, approved by
1	**Participant entry: consent and eligibility**	☐ Yes ☐ No			
2	**Delegation and training**	☐ Yes ☐ No			
3	**Sterile release to use**	☐ Yes ☐ No			
4	**Implant traceability and component reconciliation**	☐ Yes ☐ No			
5	**Instrumentation and operating-room readiness**	☐ Yes ☐ No			
6	**Operative documentation**	☐ Yes ☐ No			
7	**Imaging usability and timing**	☐ Yes ☐ No			
8	**Follow-up execution**	☐ Yes ☐ No			
9	**Endpoint data agreement**	☐ Yes ☐ No			
10	**Timeliness and closure**	☐ Yes ☐ No			
11	**External context**	☐ Yes ☐ No			
12	**Complication monitoring**	☐ Yes ☐ No			
13		☐ Yes ☐ No			
14		☐ Yes ☐ No			

Rows 13 and 14 are for checks the site added in the IMP. Use the same row number in E.

E Review of each check

One row per check in scope. Leave rows out of scope blank. Cite record IDs; do not copy their content.

#	Check and question	Execution	Evidence	Effectiveness	Finding, signal or limitation (record IDs)	Action ID · route · owner · due date
1	**Participant entry: consent and eligibility** Was consent signed on the current version before any study procedure, and eligibility confirmed by the PI before surgery?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
2	**Delegation and training** Was everyone who did a study task delegated and trained for it on that date?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
3	**Sterile release to use** Were package integrity, expiry, storage and any breach or replacement reviewed and reconciled?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
4	**Implant traceability and component reconciliation** Did implanted components match labels, lot or UDI, accountability and CRF, with discrepancies routed?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
5	**Instrumentation and operating-room readiness** Were instruments available, compatible and checked, and every substitution or malfunction recorded?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
6	**Operative documentation** Does each operative note let a reader rebuild the final configuration and intraoperative decisions?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
7	**Imaging usability and timing** Were image sets taken in window, with the required views, and usable, with a disposition for any that were not?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		

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Review of each check · checks 8–12 and added checks

E Review of each check (continued)

#	Check and question	Execution	Evidence	Effectiveness	Finding, signal or limitation (record IDs)	Action ID · route · owner · due date
8	**Follow-up execution** Were visits done in window with all endpoint-critical components, and attendance not taken as completeness?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
9	**Endpoint data agreement** Did the sampled endpoint-critical fields agree between source and CRF, with discrepancies corrected and explained?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
10	**Timeliness and closure** Were notes, data entry, queries, safety follow-up and actions closed on time, or escalated?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
11	**External context** Were safety notices, field actions and new literature on the device looked for and read against local findings?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
12	**Complication monitoring** Did complication reviews take place as planned, and were their signals sent on?	☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
13		☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		
14		☐ Complete ☐ Partial ☐ Not performed ☐ Not applicable	☐ Adequate ☐ Partial ☐ Insufficient ☐ Not applicable	☐ Effective ☐ Observation ☐ Not effective ☐ Not assessable		

F Signals across checks

Use only for a problem that does not fit in one row of E. Route: POT, SRR, CAPA, CSST, PIR, sponsor or safety route, or Not required with the reason.

Signal	Seen?	What was seen · checks (#)	Immediate containment	Route · record ID · owner
Same problem in more than one check	☐ Yes ☐ No			
Participant safety concern	☐ Yes ☐ No			
Endpoint or data reliability concern	☐ Yes ☐ No			
Device traceability or deficiency concern	☐ Yes ☐ No			
Need for closer PI oversight	☐ Yes ☐ No			
Need for CAPA or a change in the SRR	☐ Yes ☐ No			

G Action log

Every action has an owner and a due date. CAPA content stays in the CAPA log; write its ID here.

Action ID	From (# / F)	Action	Type	Owner (role)	Due date	Linked record	Status
			☐ Containment ☐ Corrective ☐ Preventive ☐ Verification				☐ Open ☐ Closed ☐ Transferred
			☐ Containment ☐ Corrective ☐ Preventive ☐ Verification				☐ Open ☐ Closed ☐ Transferred
			☐ Containment ☐ Corrective ☐ Preventive ☐ Verification				☐ Open ☐ Closed ☐ Transferred
			☐ Containment ☐ Corrective ☐ Preventive ☐ Verification				☐ Open ☐ Closed ☐ Transferred
			☐ Containment ☐ Corrective ☐ Preventive ☐ Verification				☐ Open ☐ Closed ☐ Transferred
			☐ Containment ☐ Corrective ☐ Preventive ☐ Verification				☐ Open ☐ Closed ☐ Transferred

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Outcome and certification

H Review outcome

The outcome comes only from the checks in scope.

Overall outcome	☐ Controls effective ☐ Observations only ☐ Action required ☐ Escalation required
Checks needing follow-up (#)	
Summary of findings	
PI review required?	☐ Yes, POT ID: ☐ No, reason:
Next IMC due	

I Effectiveness check of actions

An action is closed only when later evidence shows it worked, not when it is done.

Action ID	Check date	Evidence reviewed (record ID)	Result	Reviewer
			☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable	
			☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable	
			☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable	

J Reviewer certification

I confirm that this review covered the stated scope and period, that the records cited were reviewed in proportion to the checks in scope, and that every negative or uncertain rating has a finding, limitation, action or route.

☐ I did not do the work I reviewed. ☐ Exception recorded in POT:

Reviewer

Name / role:

Signature / date:

Principal Investigator (if required)

Name:

Signature / date:

File this checklist with the records it cites. A focused or triggered review does not replace the next scheduled IMC unless H changes its date.